



Care Indeed Home Health

Mock Patient Evaluation Document · Version 1.0 MOCK · Training Date
July 1, 2026

MOCK TRAINING RECORD
SYNTHETIC UAT DATA
NOT FOR CLINICAL USE
DO NOT UPLOAD TO LIVE PATIENT
CHART

EVALUATION 6 — CATASTROPHICALLY COMPLEX

Primary Diagnosis: Quadriplegia (Incomplete C4-C5 SCI) with Ventilator Dependence, Tracheostomy, PEG Tube, Neurogenic Bladder/Bowel, Autonomic Dysreflexia, and Psychiatric Crisis History

PATIENT DEMOGRAPHICS

Field	Detail
Patient Name	Darius Malik Thompson
Date of Birth	02/14/1995
Age	30
Sex	Male
Race/Ethnicity	Black / African American
Primary Language	English
Interpreter Needed	No
Marital Status	Single
Medical Record Number	CI-2025-00455
Address of Service	1122 Magnolia Court, Unit 1 (ground-floor ADA accessible), San Carlos, CA 94070
Cell Phone	(650) 555-0233 (voice-activated; patient can use with adaptive mount)
Email	darius.thompson95@email.com
County	San Mateo

EMERGENCY CONTACTS

Priority	Name	Relationship	Phone	Lives with Patient?
1	Angela Thompson	Mother	(650) 555-0234 (home) / (650) 555-0157 (cell)	Yes — primary caregiver (took leave from work; FMLA exhausted; now unpaid leave)
2	Terrence Thompson	Father	(650) 555-0235	No — divorced from mother; sees patient weekly; provides financial support
3	DeShawn Carter	Best friend / informal caregiver (trained at rehab)	(650) 555-0892	No — but stays overnight 2-3 nights/week to give mother respite

LIVING SITUATION

Field	Detail
Lives With	Mother Angela (age 58, healthy, dedicated but experiencing severe caregiver burnout — lost 20 lbs in 3 months, insomnia, crying episodes per self-report)
Residence Type	Ground-floor ADA-accessible apartment; recently modified (roll-in shower, widened doorways, Hoyer ceiling track lift installed, hospital bed with specialty mattress)
Access	Level entry (no steps); automatic door opener installed
Pets	None
DME in Home	(extensive — see separate section below)

DME INVENTORY

Equipment	Details
Ventilator (primary)	Trilogy Evo (Philips); settings: AC/VC, TV 550, RR 14, PEEP 5, FiO2 30%; patient on vent 24/7 with brief (15-30 min) trials off vent during day per pulmonology weaning protocol
Ventilator (backup)	LTV 1200; same settings; kept charged at bedside at ALL times
Suction Machine (primary)	DeVilbiss portable; bedside
Suction Machine (backup)	Manual (DeLee) suction — for power outage/emergencies
Pulse Oximeter	Continuous monitoring at night; spot-check during day
Tracheostomy Supplies	Shiley #8 cuffed trach; extra trach at bedside (same size + one size smaller — #6 — for emergency); trach care kit; suction catheters (14 Fr); NS bullets; trach ties; inner cannula replacements
Ambu Bag (BVM)	At bedside at ALL times — for emergency manual ventilation
Hospital Bed	Full-electric with Clinitron-type alternating pressure/low air loss mattress (Braden 9 — very high risk)
Hoyer Ceiling Track Lift	Installed in bedroom → bathroom path; slings x3
Tilt-in-Space Wheelchair	Permobil M5 Corpus; joystick abandoned (cannot operate) — currently using sip-and-puff control; headrest, lateral supports, hip guides, custom cushion (ROHO Quadtro Select)
Shower/Commode Chair	Roll-in shower chair with tilt function
PEG Tube Supplies	Mic-Key 20 Fr low-profile button; extension sets; 60 mL syringes; Kangaroo ePump for continuous feeds
Catheter Supplies	Foley catheter (16 Fr, 10 cc balloon); leg bags; night drainage bags; catheter care supplies; backup Foley kits x3
Bowel Program Supplies	Digital stimulation gloves, suppositories (bisacodyl), lubricant, chux pads, gloves, bowel program checklist
Communication	Eye-gaze communication device (Tobii Dynavox I-Series); voice-activated smartphone mount; Amazon Echo (voice-activated home controls); call bell (pressure-sensitive pad by left cheek — one of few volitional movements)
Emergency	Portable generator (for vent power during outage); UPS battery backup (connected to primary vent — provides 2 hrs); 911-registered address (flagged for vent-dependent patient)

PAYER INFORMATION

Priority	Payer	ID / Policy Number
Primary	Medicare Part A (SSDI)	4RB3-TW7-NX28
Secondary	Medicare Part B	Same
Tertiary	Medi-Cal (QMB — pays Medicare cost-sharing)	94062209D

PHYSICIAN INFORMATION

Role	Name	Specialty	Phone
Attending / Certifying	Dr. Raymond Choi	Physical Medicine & Rehabilitation (PM&R)	(650) 555-1020
Pulmonologist	Dr. Sarah Atkins	Pulmonary / Critical Care	(650) 555-1135
Urologist	Dr. David Moran	Urology (Neurourology)	(650) 555-1245
Psychiatrist	Dr. Nina Kovacs	Psychiatry	(650) 555-1780
PCP	Dr. Evelyn Nakamura	Family Medicine	(650) 555-0400
Wound Care	Dr. Angela Torres	Wound Care	(650) 555-1550
GI	Dr. Heather Kim	Gastroenterology	(650) 555-1320
Pain	Dr. Sanjay Patel	Pain Medicine	(650) 555-1445

F2F Encounter Date: 06/10/2025 F2F Performed By: Dr. Raymond Choi

REFERRAL / HOSPITALIZATION HISTORY

Event	Facility	Dates	Reason
Acute Hospitalization	Stanford Medical Center	03/02/2025 — 03/28/2025	Motor vehicle accident → C4-C5 burst fracture with incomplete SCI (ASIA B); anterior cervical corpectomy and fusion C4-C5 with posterior instrumentation C3-C6; tracheostomy (ventilator-dependent); PEG tube placement; Foley catheter placement
Inpatient Rehabilitation	Santa Clara Valley Medical Center SCI Unit	03/28/2025 — 06/12/2025	Comprehensive SCI rehabilitation (76 days); complicated by: (1) autonomic dysreflexia episodes x4 (urinary catheter obstruction x2, fecal impaction x1, ingrown toenail x1); (2) MRSA pneumonia (VAP) — treated; (3) Stage 3 pressure injury sacrum (developed in acute care, improved to Stage 2 in rehab); (4) psychiatric crisis — acute suicidal ideation 05/15/2025 ("I want to die, I can't live like this") requiring 1:1 observation x 72 hrs and psychiatric consultation

Start of Care Date: 06/13/2025

DIAGNOSES (30+ active diagnoses)

Priority	ICD-10	Diagnosis
Primary	G82.51	Quadriplegia, C4-C5 level, incomplete (ASIA B — sensory incomplete, motor complete below injury)
Acute	S12.491A	Other fracture of fifth cervical vertebra, initial encounter (status post surgical fusion)
Active	Z99.11	Dependence on respirator (ventilator) — 24 hours
Active	Z93.0	Tracheostomy status (Shiley #8 cuffed)
Active	J95.00	Tracheostomy complication, unspecified (history of granulation tissue at stoma — treated in rehab; monitoring)
Active	Z93.1	Gastrostomy status (PEG — Mic-Key 20 Fr)
Active	Z96.89	Presence of other specified functional implants (cervical spinal fusion hardware)
Active	G95.20	Unspecified cord bladder (neurogenic bladder — areflexic; Foley catheter dependent)
Active	K59.2	Neurogenic bowel, not elsewhere classified (areflexic bowel; requires bowel program)
Active	G90.4	Autonomic dysreflexia (episodic — MEDICAL EMERGENCY when triggered)
Active	L89.153	Pressure ulcer of sacral region, stage 3 (improved to Stage 2 in rehab; currently 3.0 cm x 2.5 cm x 0.4 cm depth, 80% granulation)
Active	L89.152	Pressure ulcer of sacral region, stage 2 (coding note: use current stage)
Active	Z87.39	History of MRSA pneumonia (ventilator-associated, 04/2025 — treated, resolved)
Active	J96.11	Chronic respiratory failure with hypoxia (SCI-related restrictive lung disease; baseline SpO2 93-95% on vent)
Active	J98.11	Atelectasis (recurrent bilateral lower lobe; requires aggressive pulmonary toilet)
Active	G89.4	Chronic pain syndrome (neuropathic pain below injury level — burning, tingling, shooting)
Active	M62.3	Immobility syndrome (contracture prevention needed — bilateral UE and LE)
Active	E44.0	Moderate protein-calorie malnutrition (albumin 2.6, weight loss)
Active	D62	Acute posthemorrhagic anemia (Hgb 9.4 — post-surgical, chronic disease)
Active	N39.0	Urinary tract infection (ESBL E. coli — treated, completing oral antibiotics)
Active	I82.4Z9	Acute embolism and thrombosis of unspecified deep vein of lower extremity (DVT right popliteal — 04/2025, on warfarin)
Active	Z79.01	Long-term anticoagulant use (warfarin — target INR 2.0-3.0)
Active	F32.2	Major depressive disorder, single episode, severe without psychotic features
Active	F43.10	Post-traumatic stress disorder (MVA-related; flashbacks, nightmares, hyperarousal)
Active	R45.851	Suicidal ideation (history — 05/15/2025; currently denying but remains HIGH RISK; safety plan in place)
Active	E87.6	Hypokalemia (PEG feed-related; supplemented)

Active	E55.9	Vitamin D deficiency (immobility, no sun exposure)
Active	M80.08XA	Age-related osteoporosis with pathological fracture (high risk — SCI-related bone loss; DEXA pending)
Active	G24.9	Dystonia, unspecified (spasticity, bilateral lower extremities — on baclofen pump evaluation list)
Active	R47.1	Dysarthria (mild — can speak with Passy-Muir valve in place when cuff deflated; limited endurance)
Active	R63.4	Abnormal weight loss

ALLERGIES

Allergen	Reaction	Severity
Latex	Anaphylaxis	LIFE-THREATENING — ALL supplies must be latex-free
Fluoroquinolones (all)	Tendon rupture (Achilles, bilateral — 2022; pre-existing on ciprofloxacin)	Severe
Tramadol	Seizure	Severe
Adhesive tape	Skin tears / MARS	Moderate

CURRENT MEDICATIONS (32 medications)

#	Medication	Dose	Route	Frequency	Purpose	Critical Notes
1	Enteral formula (Jevity 1.5)	70 mL/hr	PEG (continuous via pump)	18 hrs/day (6 AM – 12 AM; off 12 AM – 6 AM)	Primary nutrition	1890 kcal/day; supplemented with protein powder; free water flushes 200 mL Q4H
2	Baclofen	20 mg	PO (via PEG)	QID	Spasticity	Intrathecal pump evaluation pending
3	Gabapentin	600 mg	PO (via PEG)	TID	Neuropathic pain	
4	Oxycodone IR	5 mg	PO (via PEG)	Q6H PRN severe pain (max 20 mg/day)	Breakthrough pain	Controlled substance. Mother administers and tracks.
5	Warfarin	7.5 mg	PO (via PEG)	Daily (5 PM)	DVT treatment	INR monitoring 2x/week. Multiple drug interactions with PEG meds.
6	Sertraline	150 mg	PO (via PEG)	Daily AM	Depression / PTSD	
7	Mirtazapine (Remeron)	30 mg	PO (via PEG)	HS	Depression (adjunct) + appetite stimulation + insomnia	
8	Prazosin	2 mg	PO (via PEG)	HS	PTSD-related nightmares	Monitor BP (alpha-blocker — orthostatic risk less relevant in wheelchair, but can affect baseline BP)
9	Lorazepam	0.5 mg	PO (via PEG)	PRN acute anxiety/panic (max BID)	Anxiety / PTSD flashbacks	Controlled substance.
10	Oxybutynin	5 mg	PO (via PEG)	TID	Neurogenic bladder — reduce detrusor spasticity	
11	Methenamine hippurate (Hiprex)	1 g	PO (via PEG)	BID	UTI prophylaxis (recurrent)	

12	Cranberry extract	500 mg	PO (via PEG)	BID	UTI prophylaxis (adjunct)	
13	Nitrofurantoin (Macrobid)	100 mg	PO (via PEG)	BID x 7 days (through 06/20/2025)	Current ESBL E. coli UTI treatment (sensitivity-confirmed)	
14	Bisacodyl suppository	10 mg	PR	Every other day (MWF) — bowel program	Neurogenic bowel management	Administer 15-30 min before digital stimulation
15	Docusate/Senna	100/8.6 mg	PO (via PEG)	BID	Bowel program — stool softener/stimulant	
16	Polyethylene glycol (MiraLAX)	17 g	PO (via PEG)	Daily	Bowel program — osmotic	
17	Albuterol	2.5 mg/3 mL	INH (nebulizer via vent circuit)	Q4H and PRN	Bronchodilation; secretion mobilization	
18	Ipratropium	0.5 mg/2.5 mL	INH (nebulizer via vent circuit)	QID	Bronchodilation; reduce secretions	
19	Acetylcysteine (Mucomyst)	10% 3 mL	INH (nebulizer via vent circuit)	BID	Mucolytic — thin secretions	May trigger bronchospasm — give albuterol first
20	Normal Saline nebulization	3 mL	INH (nebulizer via vent circuit)	Q4H between other nebs	Secretion hydration	
21	Omeprazole	20 mg	PO (via PEG)	Daily	GERD / GI prophylaxis	
22	Warfarin — already listed above					
23	Calcium + Vitamin D	600/400	PO (via PEG)	BID	Bone health	
24	Vitamin D3	2000 IU	PO (via PEG)	Daily (in addition to calcium/D)	Deficiency	
25	Multivitamin (liquid)	15 mL	PO (via PEG)	Daily	General	
26	Zinc	50 mg	PO (via PEG)	Daily	Wound healing	

27	Vitamin C	500 mg	PO (via PEG)	BID	Wound healing	
28	Potassium chloride (liquid)	20 mEq	PO (via PEG)	BID	Hypokalemia	
29	Ferrous sulfate (liquid)	300 mg	PO (via PEG)	Daily	Anemia	Give 2 hrs apart from omeprazole and calcium
30	Melatonin	5 mg	PO (via PEG)	HS	Sleep cycle regulation	
31	Acetaminophen	650 mg	PO (via PEG)	Q6H PRN	Pain / fever	
32	PRN autonomic dysreflexia kit	Nifedipine 10 mg bite-and-swallow (via PEG) or 2% nitroglycerin paste 1 inch topical	PO or Topical	EMERGENCY USE ONLY for AD episode (SBP >150 or symptoms)	LIFE-SAVING. See AD Emergency Protocol.	Must be immediately accessible at all times — bedside, in wheelchair bag, and in bathroom.

AUTONOMIC DYSREFLEXIA — EMERGENCY PROTOCOL

THIS IS A MEDICAL EMERGENCY. Autonomic dysreflexia (AD) in this patient can be FATAL (hypertensive crisis → stroke, seizure, cardiac arrest).

Signs/Symptoms:

Sudden severe headache ("pounding")

BP >150/90 (patient's baseline is ~105/65 — ANY SBP ≥140 is concerning)

Flushing/sweating above the level of injury (face, neck, chest)

Piloerection ("goosebumps") above injury

Nasal congestion

Blurred vision

Bradycardia (may be severe — HR <50)

Anxiety, feeling of "impending doom"

IMMEDIATE ACTIONS (DO NOT WAIT FOR ORDERS):

Sit patient upright immediately (lower legs if possible) to use orthostatic effect to reduce BP.
 Loosen all restrictive clothing, abdominal binder, compression stockings.
 Check BP immediately — if SBP ≥ 150 , proceed to pharmacologic intervention.
 Identify and remove the trigger:
 Bladder (most common): Check Foley catheter — is it kinked, clamped, obstructed? Irrigate gently with 30 mL NS. If no flow, replace Foley immediately. If straight cath needed, use lidocaine gel.
 Bowel: Check for fecal impaction (apply lidocaine jelly to rectal area first to prevent worsening AD, then gently check). If impacted, disimpact carefully.
 Skin: Check for pressure injury, ingrown toenail, tight clothing, strap/belt compression, temperature extremes, tight catheter leg strap.
 Other: UTI, DVT, fracture (can occur with osteoporotic SCI bone from minor trauma), abdominal pathology.
 If trigger cannot be identified/resolved AND SBP remains ≥ 150 :
 Administer nifedipine 10 mg — bite capsule and administer contents via PEG (rapid onset).
 OR apply 2% nitroglycerin paste 1 inch to forehead or chest ABOVE the level of injury. Remove when BP normalizes.
 Monitor BP every 2-5 minutes until resolved.
 Call 911 if:
 SBP > 180 and not responding to interventions within 5 minutes.
 LOC changes.
 Seizure activity.
 Severe bradycardia (HR < 40).
 Symptoms worsen despite interventions.
 Notify physician — Dr. Choi or on-call provider.
 Document — full event documentation including time of onset, symptoms, BP readings, interventions, trigger identified, resolution time.

AD history in this patient: 4 episodes in rehab. Most recent: 05/30/2025 (fecal impaction; SBP peaked at 195/110; resolved with disimpaction + nifedipine). This patient is at HIGH RISK for recurrent AD.

CLINICAL ASSESSMENT — VITAL SIGNS AT ADMISSION

Parameter	Value	Notes
BP	108/62	Baseline for this patient (SCI hypotension — normal)
HR	64 (baseline bradycardia — SCI-related)	
RR	14 (ventilator-set) + 0-2 spontaneous over-breaths	
Temp	98.8°F	WNL
SpO2	94% on vent (FiO2 30%)	Baseline for this patient
Weight	152 lbs (69 kg) — pre-injury weight 195 lbs	Loss of 43 lbs (22%) in 3 months
Pain	7/10 neuropathic (bilateral LE — "burning, like fire ants"); 4/10 neck/surgical site	
RASS	0 (alert and calm)	

PRESSURE ULCER ASSESSMENT

Sacral Pressure Ulcer:

Parameter	Finding
Location	Sacrum/coccyx
Stage	Stage 2 (was Stage 3 in acute care; epithelialized to Stage 2 in rehab)
Dimensions	3.0 cm L x 2.5 cm W x 0.4 cm D
Wound Bed	80% granulation (beefy red), 20% epithelializing edges
Drainage	Scant serous
Periwound	Intact, mild maceration at inferior edge
Wound Care Orders	Cleanse NS; apply Mepilex Border Sacrum foam dressing; change every 3 days and PRN soiling; off-load pressure — repositioning Q2H minimum; specialty mattress continuous
Contributing Factors	Immobility, malnutrition (albumin 2.6), incontinence exposure (bowel program imperfect — occasional incontinence between programs), moisture
Braden Scale	9 (VERY HIGH RISK) — Sensory: 1 (completely limited); Moisture: 3 (occasionally moist); Activity: 1 (bedfast); Mobility: 1 (completely immobile); Nutrition: 2 (probably inadequate); Friction/Shear: 1 (problem)

Additional Skin Surveillance Areas:

Bilateral ischial tuberosities (wheelchair sitting) — currently intact; ROHO cushion in place; weight shifts via tilt every 15-30 min
Bilateral heels — currently intact; heel suspension boots in bed; float heels off mattress
Bilateral greater trochanters — currently intact; repositioning schedule
Occiput — currently intact; gel pillow
Ears — trach ties/vent tubing pressure; currently intact; pad tubing
Right lateral malleolus — mild redness, blanching — pre-Stage 1; reposition and monitor

HOMEBOUND STATUS

CONFIRMED. Ventilator-dependent quadriplegic requiring Hoyer lift for all transfers, continuous mechanical ventilation, suctioning, and attendant care for all ADLs. Unable to leave home without specialized wheelchair-accessible transport, portable ventilator, suction equipment, and at minimum one trained attendant. Taxing effort is extreme. Absences are exclusively for medical appointments (infrequent — most specialists will do telehealth visits for this patient).

ADVANCE DIRECTIVES

Document	Status
AHCD	Yes — executed 06/2025 at rehab. Mother Angela designated as primary agent. Father Terrence as alternate. Specific instructions: Patient wants full treatment including ventilator (already on vent); does NOT want CPR continued beyond 45 minutes if no response; wants comfort measures ONLY if physician determines he is in permanent vegetative state.
POLST	Yes — Section A: Full Treatment; Section B: Full treatment including intubation (already trached); Section D: Long-term artificial nutrition — yes (already on PEG feeds).
Mental Health Advance Directive	Yes — executed 06/2025. Designates mother Angela as mental health agent. Specific instructions: If experiencing acute psychiatric crisis with suicidal ideation, patient authorizes: (1) psychiatric hospitalization if recommended by Dr. Kovacs; (2) medication adjustment including antipsychotics if needed; (3) 1:1 observation; (4) does NOT authorize ECT under any circumstances.
Safety Plan (Suicide Prevention)	On file — developed 05/18/2025 at rehab with Dr. Kovacs. Copy in clinical record. Includes: warning signs, coping strategies, reasons for living (mother, friend DeShawn, desire to mentor other SCI patients), emergency contacts (988 Suicide & Crisis Lifeline; Dr. Kovacs' answering service; 911).

PSYCHIATRIC / SUICIDE RISK ASSESSMENT

This patient is at ELEVATED AND ONGOING RISK for suicidal ideation and self-harm.

Risk Factor	Present?	Details
Prior suicidal ideation	YES	05/15/2025 — active SI with plan ("I could tell my mom to turn off the vent") requiring 1:1 observation x 72 hrs
Access to lethal means	YES	Ventilator dependence itself is the means — patient could potentially direct caregiver to disconnect, or refuse ventilator if competent. Medications also accessible.
Recent major loss/life change	YES	Catastrophic SCI with quadriplegia at age 30; loss of independence, career (was a personal trainer), romantic relationship (girlfriend left), financial security, housing
Chronic pain	YES	Severe neuropathic pain — significant suffering
Depression diagnosis	YES	MDD, severe
PTSD	YES	MVA-related flashbacks, nightmares
Social isolation	PARTIAL	Mother is devoted; friend DeShawn visits; father visits weekly. BUT patient reports feeling "trapped" and "like a burden." Lost most of his social circle post-injury.
Male, young adult	YES	Demographic risk factor
Substance use	NO	No history
Current SI	NO (assessed 06/13/2025)	"Not right now. I'm not in that place anymore. But I'm not going to lie and say it won't come back."
PHQ-9	22 (severe)	
PCL-5 (PTSD)	58 (probable PTSD; >33 = clinical threshold)	
Columbia Suicide Severity Rating Scale (C-SSRS)	Ideation Type: 3 (active SI with method, no intent, no plan — at time of most recent assessment); Intensity: moderate	Baseline established for ongoing monitoring

Psychiatric Monitoring Plan:

PHQ-9 every 2 weeks (RN or MSW administered)
 C-SSRS screening every visit (RN)
 If SI reported: activate safety plan; contact Dr. Kovacs immediately; do NOT leave patient unattended; assess lethality and intent; call 988 or 911 if imminent risk
 MSW weekly visits for therapeutic support, coping skills, adjustment counseling
 Coordinate with Dr. Kovacs for medication management (monthly psychiatry telehealth visits scheduled)
 Mother and DeShawn trained on warning signs and safety plan activation

SERVICES ORDERED

Service	Discipline	Frequency	Duration
Skilled Nursing	RN	Daily x 4 weeks (vent management, trach care, wound care, PEG management, catheter care, bowel program supervision, labs, teaching, psychiatric monitoring), then 3-5x/week ongoing	Indefinite (complex, vent-dependent; anticipate ongoing need for 6+ months minimum)
Physical Therapy	PT	5x/week x 4 weeks, then 3x/week ongoing	Indefinite
Occupational Therapy	OT	5x/week x 4 weeks, then 3x/week ongoing	Indefinite
Speech-Language Pathology	SLP	3x/week x 4 weeks, then 2x/week	8 weeks (reassess)
Medical Social Work	MSW	2x/week x 4 weeks, then 1x/week ongoing	Indefinite
Home Health Aide	HHA	BID (AM + PM personal care, repositioning, equipment assist)	Indefinite
Respiratory Therapy (if available through agency; otherwise coordinated with DME vendor)	RT	2x/week x 4 weeks, then 1x/week	As needed

SKILLED NURSING FOCUS AREAS (highest complexity level)

Ventilator Management — Verify settings each visit. Assess respiratory status (breath sounds, SpO₂, ETCO₂ if available, WOB, secretion characteristics). Vent circuit change per schedule. Troubleshoot alarms. Coordinate with pulmonology for weaning trials. Emergency protocol: vent failure → disconnect from vent → manual bag ventilation with Ambu bag → assess for obstruction → call 911 if cannot reestablish ventilation. Backup vent ready at all times.

Tracheostomy Care — Trach site assessment each visit (stoma integrity, granulation tissue, infection signs). Trach care (clean stoma, change inner cannula, change trach ties) per protocol. Suctioning PRN (hyperoxygenate before/after; note: SCI patient may not cough effectively — suction may be frequent). Monitor for trach complications: tube displacement, obstruction, tracheoesophageal fistula, tracheoinnominate fistula (life-threatening — massive hemorrhage; apply direct pressure, call 911 immediately), granulation tissue, stomal infection. Passy-Muir valve application for speech during cuff-deflation trials per SLP/pulmonology. Emergency trach change protocol (spare trach at bedside).

Pulmonary Toilet — Nebulizer treatments per schedule. Chest physiotherapy (manual or mechanical insufflation-exsufflation — "CoughAssist" if ordered). Secretion management. Atelectasis prevention (position changes, deep breathing via vent sigh breaths, nebulizers). Monitor for pneumonia (changed sputum color/volume, fever, declining SpO₂, infiltrate on CXR).

PEG Tube Management — Site assessment each visit (skin integrity, granulation, leakage, buried bumper signs). Flush per protocol (30 mL water before/after meds, Q4H during feeds, before/after feed interruptions). Medication administration via PEG — verify crushability of each medication; stagger medications that interact; flush between medications. Monitor tolerance (residuals if ordered, abdominal distention, nausea, diarrhea). Coordinate with RD on formula adjustments.

Pressure Ulcer Management — Sacral wound care per orders (Mepilex Border Sacrum q3 days + PRN). Comprehensive skin assessment each visit — every bony prominence. Braden reassessment weekly. Repositioning schedule enforcement and documentation. Nutrition optimization for wound healing (protein, Vitamin C, zinc). Moisture management (incontinence care, barrier cream). Consult wound care MD for any deterioration.

Neurogenic Bladder/Catheter Management — Foley catheter care per protocol. Monitor for CAUTI signs (new fever, cloudy/malodorous urine, increased spasticity, worsening AD). Catheter change per schedule (every 4 weeks or PRN). I&O monitoring. Urine culture only if symptomatic (not routine for chronic catheter). Catheter obstruction/kinking is the #1 trigger for AD in this patient — ensure free drainage AT ALL TIMES.

Neurogenic Bowel Program — Bowel program MWF: digital stimulation + bisacodyl suppository, seated on commode chair if tolerated, consistent timing. Monitor for fecal impaction (AD trigger). Document bowel patterns. Adjust regimen per results. Fecal impaction is the #2 trigger for AD in this patient.

Autonomic Dysreflexia Surveillance — AD protocol posted at bedside, in wheelchair bag, and in bathroom. All caregivers (mother, DeShawn, HHA) must be trained and demonstrate competence in AD recognition and intervention. BP monitoring at each visit and PRN for symptoms. AD is life-threatening — this cannot be overstated.

Anticoagulation Management — Warfarin monitoring (INR 2x/week; target 2.0-3.0). Multiple drug interactions with PEG medications — warfarin dosing may fluctuate. Assess for bleeding. Coordinate with PCP/hematology.

Pain Management — Neuropathic pain assessment using appropriate scale (DVPRS or NRS). Opioid use tracking (oxycodone PRN). Non-pharmacologic interventions (positioning, distraction, music therapy, relaxation). Coordinate with Dr. Patel. Baclofen pump evaluation follow-up.

Psychiatric Safety — C-SSRS at every visit. PHQ-9 biweekly. Monitor behavioral indicators (withdrawal, refusing care, talking about death, giving away possessions, "good-bye" statements). Document and report any concerns immediately. Safety plan review with patient and caregivers. Coordinate with Dr. Kovacs.

Caregiver Training & Support — Mother Angela is the backbone of this care plan. She is experiencing severe caregiver burnout. If Angela collapses, this patient's entire home care plan fails. Priorities: (a) train Angela AND DeShawn on ALL critical skills (vent troubleshooting, manual bagging, suctioning, AD management, trach emergency, PEG management, bowel program, catheter care, medication admin); (b) assess Angela's physical and mental health at each visit; (c) connect Angela with caregiver support group and respite services; (d) explore In-Home Supportive Services (IHSS) for additional paid caregiving hours; (e) explore Regional Center/SCVMC SCI outpatient resources.

PLAN OF CARE GOALS (60-DAY CERTIFICATION PERIOD)

#	Goal	Target
1	Zero ventilator-related adverse events	08/12/2025
2	Successful vent weaning trials to ≥1 hour off-vent during daytime	08/12/2025
3	Trach downsized from #8 to #6 (per pulmonology)	08/12/2025
4	Sacral pressure ulcer fully epithelialized (closed)	08/12/2025
5	No new pressure ulcers	08/12/2025
6	Weight stable or gaining (target ≥155 lbs)	08/12/2025
7	Albumin ≥3.0 (from 2.6)	08/12/2025
8	Zero AD episodes requiring pharmacologic intervention	08/12/2025
9	Zero UTIs	08/12/2025
10	Bowel program effective (BM on program days with no incontinence between programs) for ≥80% of cycles	08/12/2025
11	INR stable within range for ≥80% of draws	08/12/2025
12	PHQ-9 decreased to ≤17 (from 22)	08/12/2025
13	Zero suicidal ideation episodes requiring crisis intervention	08/12/2025
14	Patient operates wheelchair with sip-and-puff independently in home	08/12/2025
15	Patient uses eye-gaze communication device for functional communication x 30 min sessions	08/12/2025
16	Caregiver (mother + DeShawn) demonstrate independent competence in: vent emergency, suctioning, trach care, AD protocol, PEG management, bowel program, catheter care	07/15/2025
17	IHSS application submitted and hours authorized	07/31/2025
18	Angela connected with caregiver support services	07/15/2025
19	Zero falls, zero ED visits, zero readmissions	08/12/2025
20	Patient verbalizes 3 personally meaningful goals for the next 6 months (adjustment to disability; building motivation)	08/12/2025

PATIENT SIGNATURES — EVALUATION 6

Patient Signature: _____ Date: _____

(Patient signed using adaptive grip pen holder on left hand with assistance — verified understanding verbally; patient alert, oriented, competent)

Print Name: Darius Malik Thompson

Mother / Health Care Agent Signature: _____ Date: _____

Print Name: Angela Thompson

Clinician Signature: _____ Date: _____

Patient / Representative / Conservator

Mock signature area — training only.

Evaluating Clinician

Mock signature area — training only.

Caregiver / Interpreter if applicable

Mock signature area — training only.

Date / Attestation

Mock signature area — training only.